Lotus Maternity Hospital
Department of Obstetrics and Neonatology
Pune, Maharashtra, India

Date: 8 June 2026

To: Pharmacovigilance Cell, Sunveda Life Sciences

Subject: Suspected adverse drug reactions in a mother and her newborn following sertraline exposure

Dear Sir/Madam,

We wish to report two suspected adverse reactions associated with sertraline in a mother-infant pair under our care.

MOTHER
Mrs P.R.D., 29 years old, was taking sertraline 50 mg tablets orally once daily for moderate depression. Treatment had been started by her psychiatrist on 14 October 2025, before conception, and was continued throughout pregnancy on specialist advice. She did not take any other medicine apart from routine iron and folic acid supplements.

At 36 weeks of gestation, on 22 May 2026, she presented with headache, confusion and one episode of vomiting. Serum sodium was 121 mmol/L. A diagnosis of hyponatraemia secondary to sertraline (SIADH) was made. Sertraline was withheld, fluid restriction was instituted and the sodium corrected to 134 mmol/L over four days. She has recovered.

INFANT
The baby, a boy (initials recorded as Baby R.D.), was delivered at 38 weeks on 3 June 2026 with a birth weight of 2.9 kg. Within the first 24 hours of life he developed jitteriness, a high-pitched cry, poor feeding and mild respiratory distress requiring supplemental oxygen for 36 hours. The neonatology team diagnosed neonatal adaptation syndrome attributed to in-utero exposure to sertraline. The infant did not receive any medicine himself. The symptoms are settling; at the time of writing he is feeding better but remains under observation in the neonatal unit.

Both patients remain in hospital at the time of this report.

Yours faithfully,

Dr Meera Kulkarni, MD (Obstetrics)
Dr Sanjay Pathak, DM (Neonatology)